PC-07 Pediatric VF / Pulseless VT

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Pediatric pulseless arrest with ventricular fibrillation or pulseless ventricular tachycardia. High-quality CPR and early defibrillation are central treatments.

Field flow

1. CPR + BVM	Begin continuous CPR, effective BVM ventilation, and defibrillator/AED application; minimize interruptions.
2. Shock 1	2 J/kg, then resume CPR for 2 minutes while preparing vascular access.
3. Shock 2	4 J/kg, then resume CPR immediately, obtain IV/IO, and give epinephrine 0.01 mg/kg IV/IO.
4. Refractory	Subsequent shocks at least 4 J/kg; max 10 J/kg or adult dose. Epi every 3-5 min; consider one authorized antiarrhythmic.
5. ROSC	If ROSC, use PC-08. If no ROSC, continue 2-minute rhythm/shock/medication/cause-treatment cycles.

Provider-level actions

Level	Approved actions
EMT	High-quality CPR, BVM, AED/monitor, length- or weight-based system, and immediate CPR resumption after every shock.
AEMT	All EMT care plus IV/IO, epinephrine, magnesium when indicated, and manual defibrillation only when authorized.
Paramedic	All prior care plus manual defibrillation, one authorized antiarrhythmic for shock-refractory VF/pulseless VT, EtCO2-guided CPR improvement, and reversible-cause management.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

- Do not pause CPR after a shock to check a pulse; resume until the next planned rhythm check.
- Do not delay CPR or defibrillation for IV access, medication, or intubation.
- Do not routinely use calcium or sodium bicarbonate without a specific indication.
- Use one antiarrhythmic only unless Medical Control directs otherwise.

Medication reference

Medication	Approved dose / safeguard
Epinephrine 0.1 mg/mL	0.01 mg/kg IV/IO after second shock or sooner only if rapid defibrillation is not possible; max 1 mg; repeat every 3-5 min.
Amiodarone	5 mg/kg IV/IO for shock-refractory VF/pulseless VT; max 300 mg. Select one antiarrhythmic only.
Lidocaine	1 mg/kg IV/IO for shock-refractory VF/pulseless VT; max 100 mg. Select one antiarrhythmic only.
Magnesium sulfate	25-50 mg/kg IV/IO once; max 2 g; only for torsades/polymorphic VT or suspected hypomagnesemia.

Linked pathways

AR-05 Pediatric Airway; AR-06 Pediatric Failed Airway; PC-04 Pulseless Arrest; PC-08 Post-Resuscitation; Medication Reference.

Source control

- This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.
- Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.